



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : ياسمين فواد محمد حسن
المحافظة : القاهرة الحي / المدينة : المطرية
رقم البطاقة : 27707230103524 رقم التليفون : 01063270715
العمر (تاريخ الميلاد) : 23-07-1977 المهنة : ربه منزل
العنوان : المطرية
تاريخ تسجيل الإستمارة : 26-09-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمدة 6 شهور
قسم العلاج :
الأورام

[14163]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. / د. / د. / د. / د. / د. / د. / د. / د. / د.

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ياسمين فواد محمد حسن National ID: 27707230103524	
---	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



١٠٣٥

١٠٣٥

١٠٣٥





Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



MDT form

A-Patient ID:

1- Name : ياسمين فؤاد محمد حسن

Sex:

3- Medical number:

Female

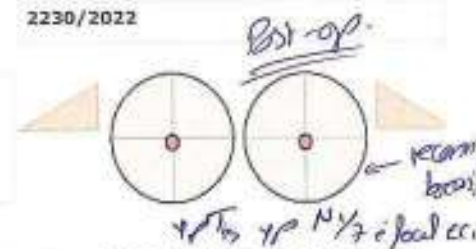
83103082

Old Med :

2230/2022

2-ID number: 27707230103524

Patient Age : 46 Y



Family history

Phone:

01061462009

BMI

REASON OF MDT

POST OPERATIVE

marital status

NRS Score

01063270715

GROUP

A

Date of MDT

mm/dd/yyyy ☐

Co morbidity

DM

Menopausal status

PREMENOPAUSAL

Clinical data

Breast Imaging :

Mets Workup :

BONE SCAN FREE

CT CAP FREE

BX:

IDC

ER:

8/8

PR:

8/8

Her 2 Neu :

+VE BY SISH

SISH :

Ki67 :

T

2

N

1

M

0

Sonomammography

PATIENT HISTORY

PT ENDED NEOADJ. TAXOTERE/CARBO 6 CYCLES+3 CYCLES HP

NO PALPABLE MASS

post NACT, lt nipple spring mastectomy +ALND:

IDC+DCIS

free nipple

free margins

YPTis , YPN1 , YPMx

MDT OUTCOME & PLAN

* PORTH to Lt CW, PCLND and axilla.

* Adjuvant TDM1 (total 14 injections).

* Adjuvant ET (AI +/- zoladex) according to menopausal status.

* IHC on surgicasl specimen.

SIGNATURE

MEDICAL ONCOLOGY	د. محمد لاشين	RADIOLOGY	د. محمد لاشين
SURGERY	د. ابراهيم لاشين	PATHOLOGY	د. محمد لاشين
Psychiatric		Nutrition	



Ministry of health
Central health laboratories
Pathology department

ImmunohistoChemistry report

Report Number: m.6224

Female :	النوع	ياسمين فؤاد محمد	المريض :
القاهرة :	العنوان	45 Y	سن :
19/11/2022 10:58 :	تاريخ استلام العينة		تاريخ اخذ العينة :
28/11/2022 13:41 :	تاريخ التقرير	مركز اورام دار السلام	لب التحليل :
		مركز اورام دار السلام	اية :

Primary Sample :

one immunostained slide prepared from parffin block labelled 8840/22

Test :

HER2 by SISH

Comment :

Technique :

sample received was stained for HER2 gene and chromosome 17 using INFORM HER2 Dual Probe Cocktail technique in benchmark GX / XT (VENTANA).

Result :

The ratio of Her 2 / chr 17 is more than 2

Referral lab :

Comment :

Conclusion :

Her 2 Status by SISH (Dual Ish Technique): Positive (amplified Her 2 / chr 17 > 2)

Pathologist:

د/أيمن رمزي

Lab Director:

د/ثرى ماهر

Patient Name	: الأستاذ / ياسمين فؤاد محمد حسن	Request No.	: 13003165
Patient No.	: 27707230103524	Request Date	: 27/03/2023
Path No.	: 961/23	Verification Date	: 10/04/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post NACT.

Nature Specimen: Left nipple sparing mastectomy specimen.

Gross Examination:

- Gross Examination:
- * Procedure: Left nipple sparing mastectomy measured 17x12x4 cm marked by sutures.
 - * Tumor extent: A central greyish white firm tumor bed site.
 - * Size: 7 x 5 cm.
 - * Skin involvement: Nipple & areola unremarkable grossly.
 - * Distance from margins: 0.1 cm from superficial margin (least) and 0.7 cm away from deep margin.
 - * Lymph nodes number and size: 7 L.Ns up to 3cm in maximal diameter.

Microscopic Examination:

- * Histological type: No residual invasive tumour (Totally burnt out tumour) in a previously diagnosed invasive duct carcinoma.
- * Ductal carcinoma in situ: Minimal residual DCIS.
- * Skin involvement: Free nipple and areola .
- * lymphovascular emboli : No.
- * perinural invasion: No.
- * Lymph node involvement: Positive for metastatic deposit in 1/7 with focal capsular infiltration .
- * Resection Margins: Free.
- * Histologic response to therapy: Partial pathological response.

Diagnosis:

- Diagnosis:
- Left breast cancer post NACT, Nipple sparing mastectomy specimen:
* No residual invasive tumour (Totally burnt out tumour) in a previously diagnosed invasive

Verified by :
DR.Menna Nabil

Printing Date : 15/04/2023 10:12:14

Page 1 of 2

Printing By : .marwa mohamed youss



Patient Name : الأستاذة / ياسمين فؤاد محمد حسن	Request No. : 13003165
Patient No. : 27707230103524	Request Date : 27/03/2023
Path No. : 961/23	Verification Date : 10/04/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 45 Y	Department : Out Patient
Referring Doc :	Registered by : .marwa mohamed youss

Pathology Report

duct carcinoma with minimal residual DCIS.

- * Free nipple and areola .
- * Lymph node were Positive for metastatic deposit in 1/7 with focal capsular infiltration .
- * Free resection Margins.
- * Partial pathological response.
- * Stage: ypTis , ypN1,ypMX.

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :
DR.Menna Nabil

Patient Name	: الأستاذ / ياسمين فؤاد محمد حسن	Request No.	: 13003165
Patient No.	: 27707230103524	Request Date	: 27/03/2023
Path No.	: 961/23	Verification Date	: 02/05/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Left breast cancer, post NACT.

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- * Procedure: Left nipple sparing mastectomy measured 17x12x4 cm marked by sutures.
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- * Lymph nodes number and size: 7 L.Ns up to 3cm in maximal diameter.

IPT:

- * Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR,HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic Examination:

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- * Ductal carcinoma in situ: Minimal residual DCIS.
- * Skin involvement: Free nipple and areola .
- *lymphovascular emboli ; No.
- *perinural invasion:No.
- * Lymph node involvement:Positive for metastatic deposit in 1/7 with focal capsular infiltration .
- * Resection Margins: Free.
- * Histologic response to therapy: Partia pathological response.

IPT:

ER: Strong nuclear reaction in 90% of tumor cells (Allred score 8/8)

Verified by :
DR.Menna Nabil

Printing Date : 20/05/2023 09:19:55

Page 1 of 2

Printed By : .marwa mohamed youss



Patient Name	الأستاذة / ياسمين فؤاد محمد حسن
Patient No.	27707230103524
Path No.	961/23
Sex	Female
Age	45 Y
Referring Doc.	

Request No.	13003165
Request Date	27/03/2023
Verification Date	02/05/2023
Financial Category	اورام دار السلام - نفقة الدولة
Department	Out Patient

Registered by .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

PR: Moderate nuclear reaction in 11% of tumor cells (Allred score 5/8).
Her-2: No membrane reaction.

Her-2: No membrane reaction in the tumor cells (score 0).
16.67-19.3%

Ki 67: 10 % nuclear reaction in tumor cells.

Positive control showed appropriate reactivity

Diagnosis:

- Left breast cancer post NACT, Nipple sparing mastectomy specimen:
- * No residual invasive tumour (Totally burnt out tumour) in a previously diagnosed invasive duct carcinoma with minimal residual DCIS.
- * Free nipple and areola .
- * Lymph node were Positive for metastatic deposit in 1/7 with focal capsular infiltration .
- * Free resection Margins.
- * Partial pathological response.
- * Stage: ypTis , ypN1,ypMX.
- * IPT:

ER: Positive (score 8/8).

PR: Positive (score 5/8).

HER-2: Negative (score 0).

Ki67: Positive in 10% of tumor cells.

Signature
Prof. Dr. Lubna Omar.
Dr. Heba Hassan.

Verified by :
DR.Menna Nabil

Printing Date : 20/05/2023 09:19:55

Page 2 of 2

Printed By : .marwa mohamed youss

Dokki Scan

Dr. Salwa Fathy
Prof. Radiodiagnosis - Cairo University
Dr. Ahmed Fathy
Consultant Radiodiagnosis
Dr. Hatem Abou El Abbas
Prof. Radiodiagnosis - Theodor Bilharz

Patient Name: Yasmien Fouad Mohamed
Prof. Dr: Center harmal
Accession: 221107053

Patient ID: 552647
Exam Date: 8/11/2022

^{99m}Tc -MDP BONE SCAN

Desired information: skeletal surveying.

Technique: Anterior and posterior Delayed Whole body bone scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc -MDP:

Report:

Delayed static phase:-

- Degenerative changes are noticed in the dorsal and lumbar vertebrae.
- There is no hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Both kidneys are visualized.

Conclusion:

- *The surveyed skeleton is free from definite osseous deposits.*

Thanks
Nuclear Medicine Unit
Dr / Huda Fathy, M.D.



Patient Name: Yasmin Fouad Mohamed Hassan	Date : 29/06/2024
Exam : ultrasound BREAST (BOTH SIDE)	Age : 47 Year(s)
Medical ID: 4230/2022	Pat. ID: 31766

RIGHT BREAST MAMMOGRAPHY WITH BILATERAL US

Indication:

Patient had left MRM with silicon implant

Technique

- right breast MLO and CC views
- Bilateral ultrasound

Report:

- The right breast is scattered fibroglandular densities of the right breast (ACR B)
- Right UOQ group of macrocalcifications.
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no right breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific right axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free left operative bed with no evidence of recurrent mass lesions.
- No sonographic evidence of implant rupture.
- Free left infra and supra clavicular regions.

Opinion:

- ✓ • Left MRM with free operative bed with no evidence of recurrent mass lesions
- ✓ • No right breast suspicious findings (BIRADS 2)

Yours truly

Dr.yomna tarek,m.Sc.

Not Approved

Patient Name: Yasmin Fouad Mohamed Hassan	Date: 06/08/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 48 Year(s)
Medical ID: 4230/2022	Pat. ID: 31766

BILATERAL MAMMOGRAM AND BREAST ULTRASONOGRAPHY REVEALED:

Indication: Follow up after left mastectomy with implant reconstruction .

- **Breast density:** Moderate dense breast tissue ... *Matching ACR C* .
- No detected definite suspicious masses, architectural distortion or pathological calcifications in the right breast.
- **Scattered right UOQ coarse macrocalcifications.**
- **Intact shape and configuration of the left breast implant .**
- Normal skin and nipple in the right breast.

By complementary bilateral breast ultrasound:

- Heterogeneous (predominantly glandular) parenchymal echo-texture.
- No detected suspicious solid or cystic masses in the right breast..
- Clear operative bed on the left side .
- No detected significant axillary lymph nodes.
- Intact shape, echogenicity and capsular outline of implant on the left side, with no signs of intra or extra capsular rupture. No detected masses.

CONCLUSION:

- **Post-operative changes of the left breast (BIRADS 2 Category).**
- **Normal study of the right breast (BIRADS 1 Category).**

MUCH OBLIGED

DR.MONA ZAKY, MD.

prof-Breast Imaging-Mansoura University

Signature Dr. Mona Zaki

06/08/2025 3:51 PM

PATIENT NAME: YASMEN FOAD MOHAMMED HASSAN	PATIENT ID #: 2022198900
GENDER: Female	ORDER #: 1781096,1781097
AGE: 45 y	DATE OF SERVICE: 15-Aug-2022 11:41 AM

Report: - all studies

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH TRIPHASIC STUDY OF THE LIVER REVEALED:**1-CHEST**

- Left breast UOQ heterogeneously enhancing parenchyma along with irregular shaped mass formation, the largest measuring about 20x26mm in axial planes.
- Left axillary (Level I & II) few enlarged lymph nodes, the largest measures about 16x28mm along its axial dimensions.
- No sizable pulmonary nodules, masses, cavitary lesions, consolidative patches or bronchiectatic changes.
- No sizable enlarged hilar or other mediastinal lymph nodes.
- No evidence of pleural effusion or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Normal appearance of the chest wall.
- A small sliding hernia.

2- ABDOMEN AND PELVIS

- The liver is of average size showing homogenous CT density with no hepatic focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Left supra renal gland small well defined nodules measures about 11x15mm in axial planes with measures HU= -2 in pre contrast study (denoting lipid rich adenoma).
- Normal CT appearance of the spleen, both kidneys, pancreas, right supra renal gland, aorta & IVC.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Bulky anteverted uterus with small hypodense intramural focal lesion

likely small fibroid.

- Left adnexal small hypodense cystic lesion likely functional cyst.
- No adnexal solid mass lesions.
- No abdominal or pelvic collections.

The revised bone window revealed:

- Mild lumbo- sacral spondylosis.
- No osseus lytic or sclerotic deposits.

OPINION:

Known case of left breast cancer revealed:

- No CT evidence of pulmonary, hepatic or osseus deposits.

Reported By,

**Dr. Kareem Moussa, MD
Dr. Salwa Yahya , MSc**

15-Aug-2022 04:44 PM

[illegible]